

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 2.9 — The Gap Inventory: What Is Not Covered

The architecture documented in Sections 2.1 through 2.8 — federal OAS, CPP, CPP-D, GIS, the Allowance, the Allowance for the Survivor, and the Alberta provincial layer of seniors programs — is a substantial public investment by historical and international standards. The architecture covers the basics of senior income support, prescription drugs (in part), dental and optical care (in part), accommodation in continuing care (with subsidization), and a defined disposable-income guarantee for the most institutionalized seniors. It does not cover everything.

This Section walks the inventory of what the architecture does not cover. The purpose is not to suggest that everything not covered should be covered — that is a policy judgment beyond the scope of this document. The purpose is to make the gaps visible, so that the working-age reader and the senior reader and the family-caregiver reader can see, in operational detail, what they will be paying for out of pocket regardless of what they receive from the public system.

Hearing aids and audiology

Hearing loss is highly prevalent among Canadian seniors. Statistics Canada estimates that approximately 47 per cent of Canadians aged 65 and over have measurable hearing loss, with the prevalence rising sharply with age. (Source: Statistics Canada — Canadian Health Measures Survey, hearing component, 2012-2015 cycle, multiple subsequent reference periods.) Hearing aids are a documented health intervention for hearing-loss-related cognitive, social, and safety impairment. The cost of a pair of mid-tier hearing aids in Canada is in the range of two thousand to seven thousand dollars, with replacement typically required every five to seven years. (Source: Canadian Academy of Audiology — Cost of Hearing Aids in Canada, 2024-2025; Hearing Life Canada — Hearing Aid Cost Guide, 2025.)

Public coverage of hearing aids in Alberta is partial. The Alberta Aids to Daily Living (AADL) program provides limited subsidy for hearing aids for eligible recipients, with a 25 per cent client co-payment up to a maximum of \$500 per year. (Source: Government of Alberta — Alberta Aids to Daily Living, hearing aids page, 2026.) The subsidy is bounded, the eligible product list is restricted, and the application process requires audiological documentation. For most Albertan seniors, the cost of hearing aids is paid out of pocket, with whatever portion of AADL subsidization applies offset against the total.

The senior who needs hearing aids and cannot afford them simply does not have hearing aids. The cognitive, social, and safety implications of untreated hearing loss compound with the financial implications. The literature on the relationship between untreated hearing loss and accelerated cognitive decline, increased social isolation, and increased fall risk is substantial. (Source: Lin, F.R., et al., "Hearing Loss and Incident Dementia," Archives of Neurology, 2011; Public Health Agency of Canada — Senior hearing health reports, 2020-2024.) The decision not to subsidize hearing aids more comprehensively at the provincial or federal level is, in operational effect, a decision to allow these compounding effects to occur.

Mobility aids beyond basic categories

Walkers, wheelchairs, and canes for basic mobility needs are covered in part by AADL with similar co-payment and product-list restrictions. (Source: Government of Alberta — Alberta Aids to Daily Living, mobility equipment page, 2026.) Power wheelchairs and scooters for seniors with more substantial mobility limitations are subject to specific medical criteria for approval and to product-list restrictions that may not

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match the senior's actual mobility profile.

Vehicle modifications for accessibility (lift conversions, hand controls, transfer seats) are not generally covered by provincial seniors programs. Home modifications for accessibility (stair lifts, walk-in tubs, accessible bathroom retrofits, ramps) may be partially funded through SHARP (Section 2.5) within the program's loan-and-grant framework, with documented eligibility constraints and project-cost ceilings. The senior whose mobility needs exceed the AADL product list or the SHARP project ceiling pays the difference out of pocket or does without.

Home care beyond AHS-provided basics

Alberta Health Services provides home care for eligible seniors based on assessed need. The service tiers — Type 1 home care, Type 2 facility-based care delivery, and Type 3 contracted private agency care — are documented in Section 2.5. (Source: *Government of Alberta — Continuing Care Overview page, 2026*.) The amount of home care provided is determined by the AHS Case Manager assessment. The literature on home care provision in Alberta consistently identifies the Case Manager-allocated hours as substantially less than the senior's documented care needs, with the gap closed by family caregivers or by private-pay home care.

Private-pay home care in Alberta runs in the range of \$25 to \$45 per hour for non-medical assistance and \$40 to \$80 per hour for nursing-trained assistance. (Source: *Sun Life Suncentral — Home Care Costs in Alberta, 2024-2025*; *Alberta Continuing Care Association — Cost data, 2025*.) For a senior requiring substantial daily assistance — bathing, transfers, meal preparation, medication management — the monthly cost of private-pay home care can easily reach \$4,000 to \$8,000 per month, well above the income of any senior whose primary income is OAS plus GIS plus the Alberta Seniors Benefit. The senior who needs more home care than AHS provides and cannot afford private-pay either receives less care than they need (with documented health and safety consequences) or moves into facility-based continuing care earlier than they would otherwise have done.

Caregiver compensation and respite

The Government of Alberta has, in successive provincial budgets, provided an Alberta Caregiver Tax Credit, with the post-2027 framework documented in the February 2026 budget consolidating two existing credits into a single Alberta Caregiver Credit worth approximately \$1,054 per year at maximum. (Source: *ATB Financial — Alberta provincial budget 2026: tax and benefit highlights, February 27, 2026*; *Government of Alberta — Budget 2026 Tax Plan*.) The federal Canada Caregiver Credit provides a parallel federal credit, with thresholds and amounts varying by the dependent's circumstances. For a family member caring for a senior or disabled adult full-time, these tax credits offset a small portion of the income forgone by reducing or ending paid employment to provide care.

Caregiver compensation programs that pay caregivers directly for the care they provide — common in some U.S. states under Medicaid waiver programs — are limited in Alberta. The Family Managed Care program under PDD provides funding directly to families to purchase services for adults with developmental disabilities, but the structure does not generally treat the family caregiver as the paid provider. The senior care equivalent does not exist in Alberta. (Source: *Government of Alberta — Family Managed Services page, 2026*.)

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The result is that the bulk of senior and disabled-adult care in Alberta is provided unpaid by family members — overwhelmingly by women, particularly by middle-aged daughters and daughters-in-law — at substantial documented cost to those caregivers' own employment, retirement savings, mental health, and physical health. (Source: Statistics Canada — *Portrait of Caregivers, Canadian Survey on Disability, 2017 and 2022*; Vanier Institute of the Family — *Caregiving in Canada, 2023*.) The system relies on this unpaid labour. The system does not financially recognize most of it.

Specialized medical equipment beyond AADL coverage

Continuous Positive Airway Pressure (CPAP) machines for sleep apnea, which is highly prevalent among older adults, may be covered in part by AADL but with significant restrictions on covered devices and supplies. (Source: Government of Alberta — *AADL Respiratory Equipment page, 2026*.) Insulin pumps for diabetes management for adults over a defined age cap may not be covered, with adult diabetics often facing out-of-pocket costs of thousands of dollars per year for pump supplies. (Source: Diabetes Canada — *Provincial coverage report, 2024*.) Specialized seating systems for severe physical disabilities, certain types of feeding tubes and supplies, and certain communication devices may have significant gaps in coverage.

Each gap is documented. Each gap has been the subject of advocacy across multiple disability-and-senior advocacy organizations. The provincial-and-federal structure addresses each gap, where it is addressed, through specific program-by-program advocacy and not through any broader principle of comprehensive coverage. The senior or disabled adult whose needs match the program inventory is supported. The senior or disabled adult whose needs fall in a gap pays out of pocket or does without.

Mental health beyond crisis services and limited covered counselling

The Canadian provincial healthcare systems cover psychiatric services delivered by physicians under the public-insurance framework. Psychological services delivered by registered psychologists are not covered under most provincial insurance frameworks; they are paid by the recipient out of pocket, by private insurance where available, or by the limited subsidization programs some provinces operate. Counselling services delivered by social workers, counsellors, or psychotherapists who are not physicians or psychologists are likewise not covered under provincial insurance.

Wait times for publicly funded mental-health services in Alberta are documented as substantial. Specialty programs for adults with serious mental illness have multi-month or multi-year waiting lists. (Source: Alberta Health Services — *Mental Health Service Standards, multiple reports*; Canadian Mental Health Association — *Alberta Division annual reports, 2022-2025*.) For many adults with serious mental-health conditions, including conditions that are AISH-qualifying, the public mental-health system delivers either crisis-stabilization services or extended waits, with limited intermediate intervention.

For seniors specifically, the gap is consequential. Late-life depression is highly prevalent, treatment-responsive when treated, and frequently undertreated. Late-life anxiety, late-onset PTSD (often related to bereavement, hospitalization, or institutional placement), and late-life substance-use disorders are documented categories of need that the public mental-health system addresses unevenly. (Source: Public Health Agency of Canada — *Mental Health of Older Adults reports, multiple years*; Canadian Coalition for Seniors' Mental Health — *Clinical Guidelines, 2014-2024*.) The senior whose mental-health needs are not met by the public system pays out of pocket for private services, depends on family for support, or goes without intervention.

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Dental beyond basic preventive

The Alberta Dental Assistance for Seniors program provides coverage for routine preventive dental care and certain restorative procedures at low-income-tested levels. (Source: *Government of Alberta — Dental and Optical Assistance for Seniors page, 2026.*) The federal Canadian Dental Care Plan, introduced in 2024-2025, provides expanded coverage for low-and-middle-income seniors who are not covered by other dental insurance, with eligibility income thresholds that exclude higher-income seniors. (Source: *Health Canada — Canadian Dental Care Plan page, 2026.*)

Major dental work — crowns, bridges, root canals, dental implants, full or partial dentures with significant complexity, surgical procedures — is covered partially or not at all, depending on the program and the procedure. For seniors whose dental health has deteriorated over decades, the cost of restorative dental work can run to thousands or tens of thousands of dollars. The senior who cannot afford this work does without it. The downstream effects on nutrition (chewing capacity), social functioning (visible appearance), and general health (oral-systemic health connections including cardiovascular and infection risks) are documented in dental and public-health literature.

Vision care beyond basic glasses and exams

The Alberta Optical Assistance for Seniors program provides coverage for routine eye examinations and basic prescription eyeglasses at low-income-tested levels with frequency limits. (Source: *Government of Alberta — Dental and Optical Assistance for Seniors page, 2026.*) Specialized vision care for age-related macular degeneration, diabetic retinopathy, glaucoma, and cataracts is covered under the provincial healthcare system at the medical-treatment level (surgery, prescription medication where covered, specialist consultations). Specialized assistive devices — high-magnification lenses, electronic visual aids, screen readers, daily-living adaptations for low vision — are covered partially or not at all by provincial seniors programs.

For seniors with low vision who are not eligible for full-scale CNIB services or who need specific equipment, the cost of low-vision adaptation is paid out of pocket or done without. The downstream effects on independence, social engagement, and safety (particularly fall risk and medication-management errors) are documented.

Transportation

Public transportation in Alberta is operated by individual municipalities. Service coverage, operating hours, and accessibility vary substantially. Most Alberta municipalities offer reduced fares for seniors and provide some level of accessible-transit service for seniors with mobility limitations. The accessible-transit services are typically pre-bookable, capacity-limited, and not available for spontaneous trips. (Source: *City of Edmonton — DATS service description; City of Calgary — Calgary Transit Access; smaller-municipality transit services, multiple cities.*)

For seniors in rural Alberta and in smaller municipalities without robust transit, transportation depends on personal vehicle, family or volunteer drivers, paid taxi or ride-share service, or doing without. The senior who cannot drive (because of medical reasons, because of vision changes, because of cognitive decline) and who lives where transit does not reach is structurally isolated from medical appointments, social engagement, grocery shopping, and most other community participation. The Alberta Seniors Benefit provides modest cash assistance that can be applied to transportation costs; the cost of consistent paid

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transportation in a non-transit-served area substantially exceeds the available assistance.

Housing beyond income support

The federal-and-provincial income-support architecture documented across Movements One and Two does not include direct housing-cost protection for most recipients. Income support is paid; housing cost is paid by the recipient from that income. In the Alberta housing market, this produces well-documented gaps:

- For AISH and ADAP recipients in private rental housing, average market rent for one-bedroom units exceeds the program's maximum benefit. The recipient pays rent from less than full income, leaving inadequate amounts for food, utilities, transportation, and other costs.
- For seniors in private rental housing, the OAS-and-GIS combination falls short of average market rent in most Alberta cities. The Alberta Seniors Benefit closes part of the gap; the gap remains.
- Community housing wait times in Alberta are documented in years; some Alberta cities have wait lists exceeding five years for accessible units.
- Alberta has no rent cap on private market increases. Recipients on fixed incomes face year-over-year rent increases that compound the affordability gap.

The campaign's *AISH Evidentiary Report Series, April 2026*, documents the housing-cost-vs-income gap for the working-age disability population in detail. The senior population faces a comparable gap, with the additional structural feature that aging in private rental housing produces continued exposure to rent increases at a life stage when income mobility is essentially zero.

What the gap inventory shows

The gap inventory is, in operational reality, a list of categories in which the senior or disabled adult is expected to provide for themselves out of an income that the federal-and-provincial system has determined to be at the floor of subsistence. The federal-and-provincial system does not cover hearing aids, advanced mobility equipment, comprehensive home care, comprehensive caregiver compensation, full mental-health treatment, full dental and vision care, comprehensive transportation, or housing-cost protection. The recipient is expected to address these needs out of OAS plus GIS plus partial provincial benefits, totaling perhaps \$20,000 to \$26,000 per year for a single senior at the floor.

The gap is not the result of program-design oversight. The gap is the result of program-design choice. The federal and provincial systems have, over decades, defined the public role narrowly — income support at a poverty floor, prescription drug coverage in part, basic dental and optical at low-income-tested levels, accommodation in continuing care with disposable-income guarantee — and have left the rest to private resources, family supports, and the recipient's own financial capacity.

For the senior and disabled adult whose financial capacity matches the public floor, the gap is structural exclusion from the categories of support that affluent seniors and adults take for granted. For the senior and disabled adult whose family resources are substantial, family transfers fill the gap (with the well-documented family-financial costs of doing so). For the senior and disabled adult with neither private resources nor family resources, the gap is the operational reality of their daily life.

Movement Two summary

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This Movement has walked the federal architecture (Sections 2.1-2.4), the Alberta provincial layer (Section 2.5), the lifecycle reassessment thread (Section 2.6), the partnership-penalty mechanism stage by stage (Section 2.7), the eight-layer "what you put in" answer (Section 2.8), and the gap inventory (this Section). The Movement has documented the architecture of senior income, the structure of disability-program capture, and the operational reality of what is and is not covered in the Canadian federal-and-Alberta system as of 2026.

The architecture is what it is. The numbers are what the published Service Canada and Government of Alberta sources document them to be. The capture mechanisms are what the program rules produce. The gaps are what the programs do not address.

Movement Three turns to the intersection chapters — the populations within the senior and disabled-adult communities for whom the architecture documented across Movements One and Two operates with specific additional structural pressures. The intersection chapters cover Indigenous seniors, 2SLGBTQ+ seniors, immigrant and racialized seniors, and senior caregivers — including disabled adults caregiving for parents and parents who are themselves caregivers entering long-term care.